LIVE CONTINUING EDUCATION · TWO SESSIONS · 2 CE

Talking to Pain

Motivational Interviewing and CBT as a hybrid toolkit for chronic pain management

MI + CBT

Dr. Jason Young · for chiropractors and manual-therapy providers

LET'S CLEAR THIS UP FIRST

What this is — and what it isn't

This is NOT...

Hiring advice. “Motivational Interviewing” is a patient-communication method — nothing to do with interviewing staff.

Psychotherapy — or outside your scope. You will not diagnose or treat mental illness today. This is psychologically *informed* chiropractic: better conversations inside the visits you already run — and your own best-practice guideline recommends exactly that.

This IS...

A communication upgrade for chronic pain care — two well-studied skill sets and one decision rule for when to use each.

And it pays everywhere: better care across the board, an easier time with your most difficult patients, stronger trauma-informed care, and home-care plans that actually get done.

Learn to talk to pain well, and every conversation in your clinic gets better.

Disclosure

Dr. Jason Young is the owner of ChiroSmarts.com, which provides continuing education for chiropractors and Oregon chiropractic assistant (CA) initial training. Today's course materials are hosted there, free of charge.

[Dr. Young has no other relevant financial relationships with ineligible companies to disclose.]

This session teaches psychologically informed practice within the chiropractic scope. It is not training to diagnose or treat mental health conditions, and it is not a substitute for licensure-level training in psychotherapy. Techniques presented are drawn from published, publicly available protocols, including the VA CBT-CP therapist manual and the motivational interviewing literature; full citations appear on the references slide.

BEFORE WE START

Get today's materials now

Scan the code — or just type:

chirosmarts.com/talking-to-pain

The full slide deck — every framework, script, and reference from today.

The handouts — the one-page summary sheet plus the full clinic toolkit: screeners, scripts, and patient worksheets.

Future CE from ChiroSmarts — join the mailing list on that page to hear about upcoming courses first.

Grab it now — then you're free to put the phone away and just be here.

How today works

Learn it, then drill it

Part one: MI — principles, then practice. Part two, after the break: CBT, same shape. The final 18 minutes puts them together.



Case-based

Three patients — Darlene, Mike, and Maria — follow us through the day. Every technique gets used on a real presentation.

Practice is most of the day

Whole-group rounds first — loud, fast, low stakes. Then partner drills. About 40 of our 100 minutes are you talking, not me.

Notes optional

Everything worth writing down is in the handout package, and I'll tell you when to photograph the one slide that matters most.

RIGHT NOW · 90 SECONDS

Find your partner

Someone near you — **preferably someone you haven't met yet.** Introduce yourselves. You'll work together twice today.

Why a stranger? With a friend, you'll chat. With a stranger, you'll have to actually use the skills — and that's the point. Every patient starts as a stranger too.

By the end of this afternoon, you will be able to

1 · Identify

Recognize which patient presentations and clinical moments call for MI, which call for CBT, and when to blend them.

2 · Apply

Deliver the skills efficiently across real encounter lengths — from a 10-minute adjustment visit to a 60-minute consult.

3 · Recognize

Spot yellow flags, ambivalence, and readiness cues — the signals that drive your choice of approach.

4 · Demonstrate

Practice at least two brief techniques from each skill set today — reflections and rulers; reframing and pacing.

This afternoon, in two parts

Time	Block	Mode
2:10	The patient we dread — and the 2-Question Triage	Intro + frame
2:18	Motivational Interviewing — the principles	Teach
2:35	MI practice — whole group, then partners	Practice
3:00	Break	30 minutes
3:30	CBT for chronic pain — the principles	Teach
3:48	CBT practice — Mike as a group, then partners	Practice
4:02	The hybrid — the triage in action, real examples	Demos + close

Teal = MI · Gold = CBT — the colors track the two skill sets all day · we finish at 4:20

LET'S BE HONEST

The patient you dread



Darlene, 58 · LBP, nine years · *“My spouse made me come.”*

You know them on sight: not motivated to change — and wanting one of three things you cannot deliver:

The one-step fix

“Just crack it and make it stop.” A structural answer to a nine-year problem.

Endless passive care

Treatment done *to* them, forever. No homework, no role of their own.

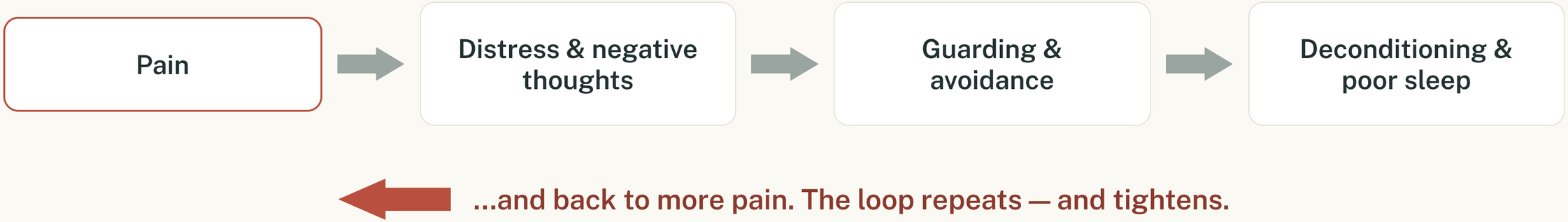
The pity party

Agreement that it's hopeless — and that nothing could ever be expected of them.

We can't deliver any of the three. What we *can* do is change the conversation — and there is a map.

WHY WORDS CHANGE OUTCOMES

Chronic pain is a cycle, not just a symptom



Fear-avoidance, catastrophizing, deconditioning, disrupted sleep, mood, and social reinforcement all keep the loop spinning. Adjust the joint and you address one arc of the circle. Talk well, and you can reach the rest.

WHY WORDS CHANGE OUTCOMES

The evidence, honestly stated

CBT · Cochrane 2020

75 RCTs · 9,401 adults

Small but reliable reductions in pain, disability, and distress versus usual care — from a conversation-based intervention.

MI · meta-analysis 2016

7 RCTs · 962 adults

Improved short-term adherence to pain treatment. MI's target is engagement — not analgesia. Effects on pain itself are less certain.

Your guideline · 2020

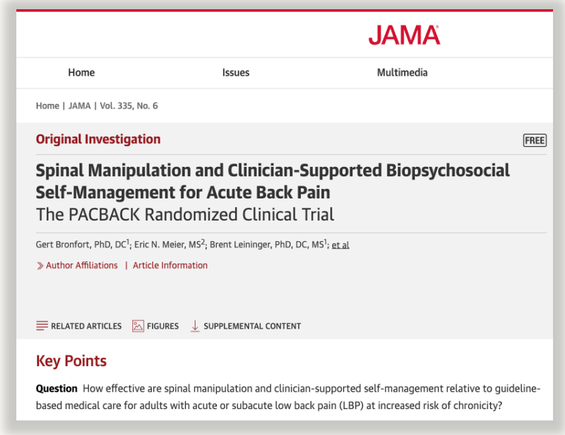
Chiropractic best practice

Chronic MSK pain belongs in a biopsychosocial model. Self-management first; CBT-style strategies explicitly recommended; screen for yellow flags.

Williams 2020 (Cochrane CD007407) · Alperstein & Sharpe 2016 · Hawk 2020

WHY WORDS CHANGE OUTCOMES · HOT OFF THE PRESS

PACBACK (2026): the head-to-head we've been waiting for


JAMA

Home Issues Multimedia

Home | JAMA | Vol. 335, No. 6

Original Investigation FREE

Spinal Manipulation and Clinician-Supported Biopsychosocial Self-Management for Acute Back Pain
The PACBACK Randomized Clinical Trial

Gert Bronfort, PhD, DC[1]; Eric N. Meier, MS[2]; Brent Leininger, PhD, DC, MS[1]; et al

Author Affiliations | Article Information

RELATED ARTICLES FIGURES SUPPLEMENTAL CONTENT

Key Points

Question How effective are spinal manipulation and clinician-supported self-management relative to guideline-based medical care for adults with acute or subacute low back pain (LBP) at increased risk of chronicity?

The design

1,000 adults with acute/subacute low back pain at **medium-to-high STarT Back risk** of going chronic. Four arms, 12-month follow-up: SMT alone · clinician-supported **biopsychosocial self-management** (skills like today's) · both combined · guideline medical care.

The findings

- Self-management arms beat medical care on 12-month disability and chronic-pain impact — **~64–67% achieved ≥50% improvement vs ~55%**
- **SMT alone did not** separate from guideline medical care on those outcomes
- **~76% of the benefit ran through psychosocial change** — fear, catastrophizing, self-efficacy

This is not an argument against your hands. It's evidence that *the conversation is an active ingredient* — the "clinician-supported self-management" that moved the needle is exactly the MI + CBT toolkit you'll learn today.

Bronfort et al., JAMA 2026 & JAMA Intern Med 2026 — the PACBACK randomized clinical trial

WHO NEEDS THIS APPROACH?

Screen, don't guess

Your dread is a decent detector — but two minutes of paper is a better one. Three free tools that flag the patients who need the biopsychosocial-heavy approach:

STarT Back

9 items for low back pain (Keele University). Stratifies low / medium / **high** risk. High risk = heavy psychosocial load — fear, catastrophizing, anxiety, low mood.

High-risk patients are exactly who MI + CBT are for.

PHQ-4 on intake

Ultra-brief: 2 depression items (PHQ-2) + 2 anxiety items (GAD-2). Score of 3+ on either pair is a flag. Fits on your new-patient paperwork.

The cheapest yellow-flag detector there is.

PHQ-2 → PHQ-9

PHQ-2 at 3+ → give the full PHQ-9 (0–27; 10+ = moderate depression, a referral conversation).

Item 9 (thoughts of self-harm) positive = same-day action. Know your pathway first.

A high psychosocial score doesn't mean “refer everyone out.” It means: for this patient, the conversation IS the treatment — MI and CBT move to the front.

Hill et al. 2011 (STarT Back) · Kroenke et al. (PHQ measures) · Hawk et al. 2020 — screen for yellow flags

MOTIVATIONAL INTERVIEWING

MI is for the patient who *won't.*

It answers: “Will this patient engage?” Use it when the problem is willingness.

COGNITIVE BEHAVIORAL THERAPY

CBT is for the patient who *can't.*

It answers: “Does this patient have the skills?” Use it when the problem is ability.

Most patients are both — at different moments. That is why you need the hybrid.

THE MAP FOR TODAY

The 2-Question Triage

1 · Are they willing?

The motivation question. When the answer is no, you need **Motivational Interviewing** — hour one.

2 · Are they able?

The skills question. When the answer is no, you need **CBT** — hour two.

MI is for the patient who *won't*. CBT is for the patient who *can't*. The dreaded patient is usually both — and the last 20 minutes shows you how to run the two together.

PART ONE · 2:18 – 3:00

Motivational Interviewing

The tool for the patient who isn't **willing**.

The principles — spirit, OARS, change talk. Then the back half of this session is you doing the talking.

MOTIVATIONAL INTERVIEWING

Spirit before technique

Partnership

Two experts in the room. You know spines; they know their life.

Acceptance

Ambivalence is normal, not a character flaw to correct.

Compassion

Their goals lead — not your care plan's schedule.

Evocation

The reasons to change are already in the patient. Draw them out.

Every technique on the next slides fails if it is delivered as a trick. The stance is the intervention.

PACE

Miller & Rollnick, Motivational Interviewing, 4th ed.

MOTIVATIONAL INTERVIEWING · A WARNING

This isn't Inception



MI is not a set of tricks for planting *your* plan in the patient's head. Patients smell manipulation instantly — and it costs you the one thing MI runs on: trust.

Manipulation

- Steer them to the conclusion you already picked
- Use their words as leverage for *your* care plan
- Win the argument

Motivational Interviewing

- Evoke *their* reasons — which may surprise you
- Help them discover the path *they* believe is best
- Stay genuinely open to where that leads

The patient is the expert on their own life. The only path a patient will actually walk is the one they chose themselves.

MOTIVATIONAL INTERVIEWING

The righting reflex

Your trained urge to **fix, lecture, and prescribe** the moment you spot the problem. It feels like helping. Watch what it produces:

You: “You really need to be doing your walking program every day.”

Patient: “Yes, but by the time I get home from work I can barely stand.”

You: “Even ten minutes would help.”

Patient: “You don't understand my schedule.”



Pushback is not a patient trait. It is a signal you're using the wrong tool.

MOTIVATIONAL INTERVIEWING

OARS — the four core skills



	Skill	Sounds like
O	Open questions	“What has this pain been costing you?” — not “Pain still an 8?”
A	Affirmations	“You came back even though last week was rough. That says something about you.”
R	Reflections	“So the mornings are the hardest part.” The workhorse — next slide.
S	Summaries	“Let me pull together what you've told me…” — hand their change talk back as a bouquet.

All four fit inside a normal chiropractic visit. None of them require extra time — only different sentences.

MOTIVATIONAL INTERVIEWING

Reflections are the workhorse



Target ratio: **two reflections for every question.** Patient says: *“I know I should exercise, but I'm scared it'll flare me up again.”*

Simple reflection

“You're worried exercise will set you back.”

Restates the content. Safe, keeps them talking.

Complex reflection

“Part of you is ready to move — and part of you remembers the last crash.”

Names the feeling under the words. This is where MI earns its keep.

A reflection is a statement, not a question. Drop your voice at the end. It says: *I heard you* — and it invites the next sentence.

MOTIVATIONAL INTERVIEWING



Every ambivalent patient speaks two languages

Change talk — arguing for change

“I can't keep living like this.”
“Maybe if the walks were shorter…”
“I miss playing with my grandkids.”

Sustain talk — arguing for staying put

“I've tried everything already.”
“Exercise just flares me up.”
“I don't have time for any of this.”

Both voices live in the **same patient** — often in the same sentence: *“I know I should walk, **but** it always ends badly.”* That's not resistance. That's ambivalence, and it's the normal starting state of change.

Why it matters: people believe what they hear themselves say. The voice that talks the most in your visit is the one that wins the week after.

MOTIVATIONAL INTERVIEWING

Listen for change talk — then water it

Preparatory — DARN

- **D**esire — “I want to get back to the golf course.”
- **A**bility — “I could probably walk after dinner.”
- **R**easons — “My granddaughter wants me on the floor with her.”
- **N**eed — “I can't keep missing work like this.”

Mobilizing — CAT

- **C**ommitment — “I'm going to start this week.”
- **A**ctivation — “I already dug out my walking shoes.”
- **T**aking steps — “I walked twice since I saw you.”

When you hear CAT language, the window for skills is open.

Your job: notice and water the change talk. Never argue with the sustain talk.



MOTIVATIONAL INTERVIEWING · FAST TOOLS

The rulers: motivation in 60 seconds

Step 1: “On a 0-to-10 scale, how important is it to you to get back to golf?”

Step 2: Patient says “5.” You ask: **“Why a 5 — and not a 2?”**

Step 3: The patient now lists their own reasons to change. Reflect them back.

The follow-up question IS the technique. Asked the other way — “why not a 9?” — the patient argues for staying stuck.

Second dial: confidence. “How confident are you that you could walk ten minutes a day?” Low importance → work on why it matters. Low confidence → shrink the task.

MOTIVATIONAL INTERVIEWING · FAST TOOLS

Elicit — Provide — Elicit: advice without the argument

1 · Elicit

“What do you already know about why we pace activity?”

2 · Provide

“Can I share what I'm seeing in your case?” — then one small piece, not the lecture.

3 · Elicit

“What do you make of that?”

Asking permission before advising sounds almost too simple — and it roughly doubles the odds the advice is heard rather than deflected. This move is the bridge between the two skill sets: **every CBT technique this afternoon gets delivered inside this wrapper.**

MI PRACTICE · WHOLE GROUP · ROUND 1 OF 3

Name that voice

How it works

I read a real patient statement. You call out — loud, fast, no overthinking:

“Change!” **“Sustain!”** **“Both!”**

The trap to listen for

“I know I should, **but**…” — both voices, one sentence. When you hear “Both!”, name where the hinge is.

Why it matters: you can't water the change talk until your ear picks it out automatically.

MI PRACTICE · WHOLE GROUP · ROUND 2 OF 3

The reflection relay

How it works

I read a patient statement. Someone offers a **two-part reflection**. We polish it together, then the next statement goes to a new voice.

Two shapes that always work

“You feel ___ because ___.”
“Part of you ___, and part of you ___.”

Statement, not question — drop your voice at the end. A slightly wrong reflection still wins: the patient corrects you and tells you more.

MI PRACTICE · WHOLE GROUP · ROUND 3 OF 3

EPE, together — meet Maria



Maria, 52 · knee & low back OA

“Walking is a joke for someone my size. I'm too fat to exercise. Just fix my back so I don't have to.”

Elicit — the room proposes the opening question. What does *she* already know or believe?

Provide — permission first. One piece of information. Not a lecture wearing a disguise.

Elicit — “What do you make of that?” The win: Maria names her own next step.

MI PRACTICE · PARTNERS · 12 MINUTES · REJOIN YOUR PARTNER

Your turn — the full toolkit

Prompt 1 · “No time”

Chronic neck pain. “Between work and the kids, I don't have time for exercises.”

Prompt 2 · “Just crack it”

“I don't want homework. Adjust me like my old chiro did, and I'll see you next week. Forever.”

Prompt 3 · your own

Play your real dreaded patient. Give your partner 15 seconds of backstory first.

Clinician's checklist: open question → 2+ reflections → one ruler + “why not lower?” → EPE with permission. **No unrequested advice.**

5 minutes each way → 2-minute debrief: *“Where did you feel the righting reflex pull hardest?”*

3:00 · 30 MINUTES · BACK AT 3:30

Intermission

Part one: the tool for *willing*. Part two: the tool for *able*. Stretch — we stop *before* the attention flares.

Haven't grabbed the materials yet? chirosmarts.com/talking-to-pain

PART TWO · 3:30 – 4:02

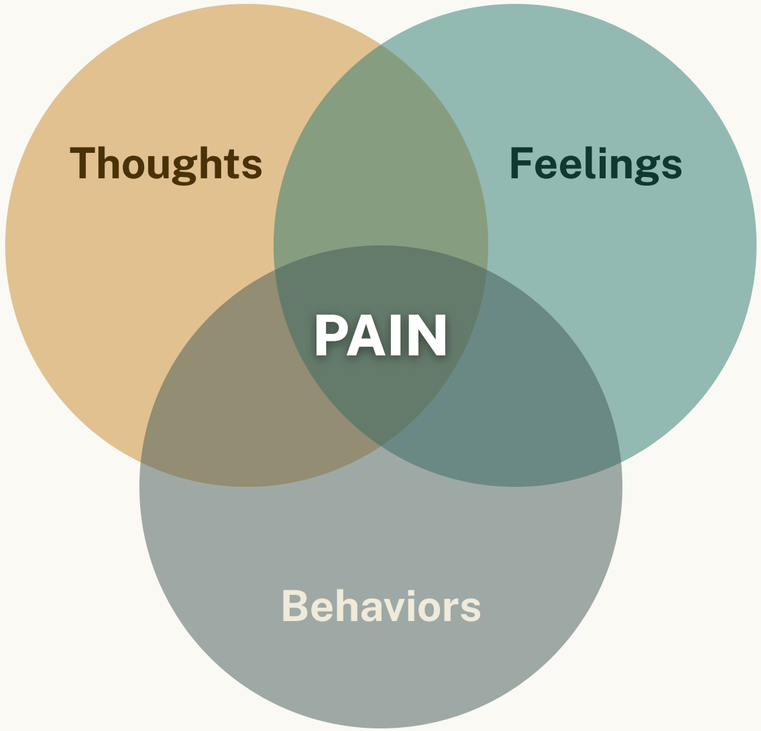
CBT for Chronic Pain

The tool for the patient who isn't **able**.

Same shape as part one: the model and five skills, then Mike as a group, then you practice.

CBT FOR CHRONIC PAIN

The model: pain lives in the overlap



The chronic pain experience sits where thoughts, feelings, and behaviors intersect. “If it hurts, I'm damaging my spine” → fear → bracing and avoidance → deconditioning → more pain — and the thought feels more true than ever.

CBT's move: shift any one circle — the thought, the behavior — and the overlap shrinks. You don't have to fix everything; you have to move something.

You can tell this story with unusual credibility: **you feel the guarding under your hands.** Chronic pain is a chronic stressor — sympathetic arousal and muscle tension amplify the very signal the patient fears.

CBT FOR CHRONIC PAIN · TRANSPARENCY

Say the name. It's not a secret technique.

> *“Some of what we'll do together comes from a framework called cognitive behavioral therapy — CBT. It's one of the best-studied tools there is for people who want change. And change can even mean changing the cycle of chronic pain.”*

Naming it builds trust

Transparency is the opposite of manipulation. “Here's the tool and here's why” treats the patient as a partner, not a target.

Naming it multiplies you

A patient who knows the word “CBT” can find their own resources — workbooks, apps, therapists — and keep building between visits.

When we help patients find motivation and autonomy, we are giving them power over their pain.

CBT FOR CHRONIC PAIN

Five skills worth owning

1 · Reframe

Pain education: hurt ≠ harm. The alarm-system story.

2 · Pacing

Activity by the clock, not by the pain. Graded walking.

3 · Thought work

Catch – check – change the hot thought.

4 · Relaxation

Breathing you can teach on the table in 3 minutes.

5 · SMART goals

Turn “I should walk more” into a plan you can check on Thursday.

These five are not the whole of CBT — the full toolbox is much bigger (activity scheduling, sleep work, exposure, problem-solving, and more). They're the five we're focusing on today: compressed from the VA's 12-session CBT-CP protocol to what fits a chiropractic practice.

Rule of thumb: **one skill per patient at a time.** A patient who leaves with one tool uses it; a patient who leaves with five uses none.

CBT SKILL 1 · REFRAME

Hurt ≠ harm

The alarm-system story

“Acute pain is a working smoke alarm — it means fire. After months or years, the alarm's wiring gets sensitive. Now it can go off for burnt toast. The noise is real. The house is not burning down.”

Why it matters

The belief “pain = damage” drives fear-avoidance — the engine of the chronic pain cycle. Loosen the belief and movement becomes thinkable again.

Hybrid moment #1: never deliver this as a lecture. Wrap it in Elicit–Provide–Elicit — “What's your understanding of why it still hurts?” → the alarm story → “What do you make of that?”

CBT SKILL 2 · PACING

Activity by the clock, not by the pain

Boom – bust

Good day → mow the whole lawn → three days on the couch → fear grows → baseline shrinks. Over-activity is as costly as avoidance.

Time-based pacing

Find the tolerable dose → stop at the timer, *before* the flare → increase by small planned steps each week. Endurance grows; flares shrink.

Default prescription: a **graduated walking program** with a written log. Start below what they can do on a bad day — the first goal is a streak, not a distance.

VA CBT-CP protocol, sessions 4 & 11 — pacing, activity logs, flare planning

CBT SKILL 3 · THOUGHT WORK

Catch — check — change

Catch

Name the hot thought: *“If it hurts, I'm damaging my spine.”*

Check

Examine it: “You were sore after walking Tuesday — what happened by Thursday?”

Change

A workable alternative: *“Sore but safe.”* Believable beats cheerful.

Yellow flag #1 — catastrophizing: “It's ruined,” “it will only get worse,” “I can't stand this.” The strongest psychological predictor of poor outcomes — and the thought pattern this skill exists to interrupt.

CBT SKILL 4 · RELAXATION

Three minutes, on the table

Diaphragmatic breathing — teach it during care

- One hand on chest, one on belly — “make the bottom hand move.”
- In through the nose ~4 counts, out slowly ~6. The long exhale is the lever.
- You'll feel the paraspinals let go under your hands — narrate it: “That softening? That's yours. You just did that.”

Home practice

Progressive muscle relaxation and guided imagery as handouts or free apps. Daily practice is what converts a trick into a skill.

Rationale for the skeptic: chronic pain is a chronic stressor. Bracing and sympathetic arousal add real, physical tension on top of the original problem. Active relaxation is not lounging — it is training the nervous system out of the guard position.

VA CBT-CP protocol, sessions 5–6 — relaxation training

CBT SKILL 4 · RESOURCES

Relaxation: what to send home

Techniques to prescribe

- **4-in / 6-out breathing** — the long exhale is the lever
- **Box breathing** — 4 in · 4 hold · 4 out · 4 hold
- **Progressive muscle relaxation** — tense 5s, release 10s, feet to jaw
- **Guided imagery** — one peaceful place, all five senses

Free and findable

- **VA CBT-CP patient handouts** — breathing, PMR, and imagery scripts, public and free
- **Mindfulness Coach** (VA) — free app, no account needed
- **Insight Timer** — large free library of guided sessions
- **CBT-i Coach** (VA) — for pain-disrupted sleep

Prescribe it like exercise

- Name the dose: once a day, five minutes
- Practice it once in-office first — on the table
- Have them log it, like the walking program
- Review at the next visit — SMART-shape it

Let the patient pick the modality — breath, muscle, or imagery. Choice is an MI move, and the chosen tool is the practiced tool.

VA CBT-CP protocol, sessions 5–6 — relaxation training and home practice

CBT SKILL 5 · SMART GOALS

From good intentions to a plan

“I should really start walking more.”
A wish. Nobody can fail at it — or succeed.

“I'll walk 10 minutes after dinner — Monday, Wednesday, Friday — with my wife, starting tonight. Log on the fridge; we review it Thursday.”
A plan. Checkable at the next visit.

Specific
What, where, with whom — not “get active.”

Measurable
You can count it: minutes, days, laps.

Achievable
Below bad-day capacity. A streak beats a distance.

Relevant
Built from what pain took from *them* — their own reasons.

Time-bound
Starts on a date. Reviewed at the next visit.

The patient supplies the goal (that's MI) — **SMART supplies the shape** (that's CBT). Your care plan is the built-in review loop.

VA CBT-CP protocol, session 3 — goal setting

CBT PRACTICE · THE CASE

Mike, 45 — warehouse worker



The presentation

- Flared low back pain; off work three weeks
- “My back is destroyed. If I do any more lifting my disc will explode.”
- Good days: cleans the whole garage. Then three days flat.
- Wants to get back to work. Terrified of re-injury.

Warm-up — call them out

- The hot thought?
- The behavior pattern?
- Willing? Able?

CBT PRACTICE · WHOLE GROUP · CALL AND RESPONSE

The five skills, live on Mike



I name a skill — you tell me how you'd use it with Mike. One good example each, then we move on. Say it as you'd say it *to him*.

Reframe? **Pacing?** **Thought work?** **Relaxation?** **SMART goal?**

The bar: patient words, not textbook words. “Sore but safe” beats “we'll address your catastrophizing.”

CBT PRACTICE · PARTNERS · 7 MINUTES · SAME PARTNER

Teach one skill — in three minutes

Prompt 1 · all-or-nothing golfer

"If I can't play a full 18, what's the point of any of it?" Likely skills: **pacing** or a **SMART goal**.

Prompt 2 · "hurt = harm"

"I can hear the joints grinding. I know they are getting shredded." Likely skill: the **alarm reframe**.

Prompt 3 · the doom-scroller

Post-MVA neck pain: "I read online this never heals." Likely skill: **catch–check–change**.

Clinician's checklist: pick ONE skill → ask permission → teach it in under 3 minutes → the patient chooses the micro-homework — **then shape it SMART together.** Deliver it all inside EPE: the MI chassis carrying CBT cargo.

3 minutes each way → 1-minute debrief: *"Was your homework SMART — and who picked it?"*

THE FINALE · 4:02 – 4:20

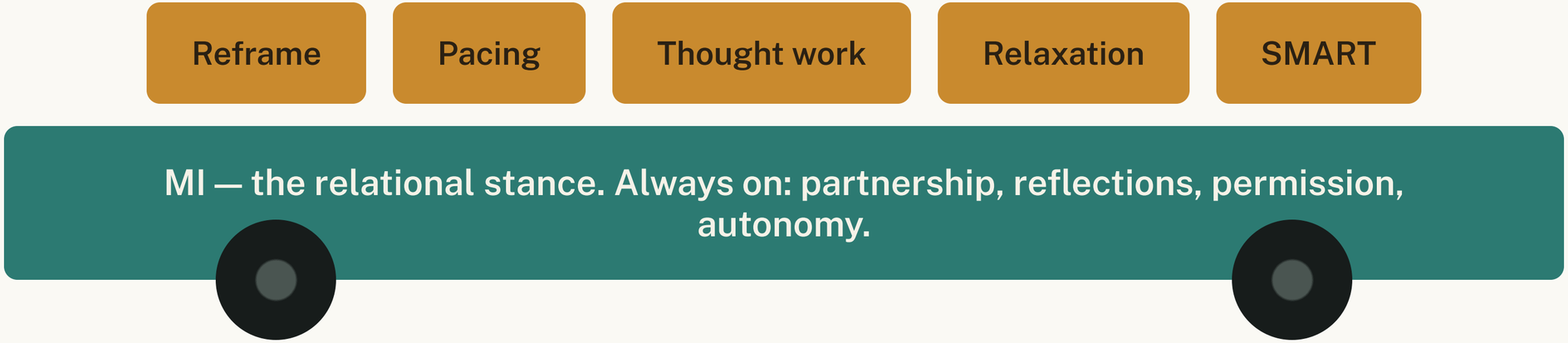
The triage, in action

You've had the map since the first ten minutes. Now you have both toolkits — watch it run on a real patient.

Willing? Able?

THE HYBRID

MI is the chassis. CBT is the cargo.



CBT skills fail when motivation is absent. MI alone builds motivation but gives the patient nothing to *do*. Ride the teal layer everywhere; unload one gold box only when the patient is ready to receive it.

Naar & Safren, Motivational Interviewing and CBT: Combining Strategies for Maximum Effectiveness

The 2-Question Triage — the full map

1 · Are they willing?
Listening for: “yes, but” · blame · hopelessness · sustain talk

No →

MI mode
Reflect. Roll with it. Rulers. Evoke. Do **not** teach skills yet.

Yes ↓

2 · Are they able?
Listening for: crashing · catastrophizing · avoiding · not sleeping

No →

CBT mode
Pick **one** skill. Teach it via Elicit–Provide–Elicit. Micro-homework.

Yes ↓

Willing and able → reinforce, progress the plan, affirm the identity: “You're becoming someone who manages this.”

Neither? MI first, always. Motivation before mechanics.

THE HYBRID

Switching cues — changing channels mid-conversation

Patient argues back, goes quiet, or “yes, but”s you	**Switch to MI.** You drifted into lecture. Reflect, back off, restore the partnership.
“So what do I actually *do*?” — or change talk peaks (CAT language)	**Switch to CBT.** The window is open. Deliver one skill, now, via EPE.
Homework didn't happen	**MI first.** “What got in the way?” — never “you need to be more compliant.” Resist the righting reflex.

DEMO · WATCH THE CHANNELS CHANGE



Darlene, part 1 — “My spouse made me come”

Darlene: “Honestly, I'm only here because Bill nagged me. Nothing's worked in nine years. No offense, but you won't be different.”

sustain talk → MI mode: reflect, don't defend

DC: “Nine years of trying things that didn't stick — you'd be crazy *not* to be skeptical.”

Darlene: “Exactly. I mean… I can't keep living like this either. I missed my granddaughter's recital last month.”

change talk (need + reason) → water it

DC: “The pain is starting to cost you the things that matter most. On a 0-to-10, how important is getting some of that back?”

Darlene: “…An 8. But I've got no faith anything works.”

Importance: 8. Confidence: low. Diagnosis by triage → willing is arriving; **able** is the gap. Part 2…

DEMO · WATCH THE CHANNELS CHANGE



Darlene, part 2 — the window opens

DC: “An 8 on importance, and almost no confidence. What would need to be true for the confidence to come up even a little?”

Darlene: “I'd need to see something actually change. Even something small. So what would you even do?”

“what would you do?” → the window — switch to CBT mode, via EPE

DC: “What do you already know about why long-term pain behaves differently from a fresh injury?” (elicit) — the alarm story, 45 seconds — “What do you make of that?” (elicit)

Darlene: “Nobody's ever explained it like that. So the pain's real but the alarm's too loud… okay. What's small enough that even I can't fail at it?”

DC: “You tell me — what's a walk so short it's almost silly?” (she picks 5 minutes → written on a card → “see you Thursday”)

Two channel switches, one skill delivered, one micro-commitment — in under four minutes of talking.

PRACTICAL APPLICATION

Shrink the dose, not the method

10-minute visit

- 1 open question while assessing
- 1–2 reflections during hands-on time
- 1 micro-skill on the table
- “What's the one thing you'll try before Thursday?”

25–30 minute visit

- 5 min MI — rulers, locate readiness
- 10 min — teach ONE skill, practice it in-room
- 5 min — “what could get in the way?”
- 5 min — summarize in their words + written homework

60-minute consult

- Engage — their story, uninterrupted
- Focus — “pain, sleep, activity, stress: where do we start?”
- Evoke — rulers, change talk
- Plan — one skill + graded walking + commitment

Minimum effective dose, every encounter: one MI move + one CBT move. And the math: twelve 10-minute conversations across a care plan ≈ two full therapy sessions of contact. The care plan IS the protocol.

GUARDRAILS · ~4:12

Your lane — and when to refer

Yours

Psychologically informed chiropractic: conversational tools that support self-management of a condition you already treat. Screening, noticing, referring.

Refer when you see

- Moderate–severe depression or anxiety (screen: PHQ-2 / PHQ-4)
- PTSD symptoms · trauma disclosure beyond your training
- Any suicidal ideation — know your local crisis pathway *before* you need it
- Substance use concerns
- Psych factors still blocking progress after your best MI/CBT effort

Build your bench now: a pain psychologist, a CBT-trained therapist, and a telehealth option for rural patients. A warm handoff (“I know someone excellent for exactly this”) keeps the patient — and the relationship.

Referral guidance per Hawk et al. 2020 — refer when psychological factors are an obstacle to progress

CLOSING THE LOOP — ON YOU

On a 0–10: how important is using one of these skills Monday morning?

...and why isn't your number lower?

Now turn to your neighbor and finish this sentence: **“With my first chronic pain patient next week, the one technique I'll use is...”**

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Thank you

Questions?

MI for the patient who won't

CBT for the patient who can't

Dr. Jason Young
dryoung@yourbodyofhealth.com
ChiroSmarts.com

Slides + handout + CE mailing list
chirosmarts.com/talking-to-pain